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# GSK plc (GSK) — Valuation, Forecast & Positioning

*A biopharma-only GSK, cheap versus its peers and creating value on returns — but priced for the 2028-29 HIV patent cliff it must grow through. Where it should trade, what a currency artifact hides, and where the price could go.*

**Ticker** GSK **Sector** Health Care **As of** 2026-08-01 · $51.69 **ROIC** 22.2% vs WACC ~8%

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**Governance & guardrails.** Research only — **not investment advice** (Ethics 3.30). Valuation is mechanical peer arithmetic; forecasts are scenario anchors with wide, partly unquantified uncertainty. Data via yfinance; historical multiples reconstructed from annual statements. Forecast on a near-random-walk series (ADF p≈0.847); Monte Carlo embeds recovery-period drift. Qualitative sections reflect analyst knowledge and must be re-verified.

## EXECUTIVE BRIEF

**GSK (NYSE: GSK, the USD ADR) trades at $51.69, almost exactly on its 200-day average and its own EV/EBITDA sensitivity base ($52), after a round-trip from a $35 low to a $60 high over the past year. On the one valuation lens that is currency-clean — forward P/E of 10.1x against a large-cap-pharma peer median of 14.6x, a 31% discount — GSK is genuinely cheap, and it earns a 22.2% ROIC against a 7.5% cost of capital (a wide +14.7-point spread that ties Merck for the highest in the peer set). So why the discount? Because the market is pricing the loss of exclusivity on dolutegravir, the ViiV HIV franchise that is GSK's single biggest earner (HIV GBP 7.7B of GBP 32.7B group sales), around 2028-29: the Mauboussin warranted multiple at just 3% durable growth is 19x, versus the 10.1x the market actually pays, which backs out to roughly -4.4% implied durable growth. The market**

**is explicitly pricing decline. The post-Haleon (July 2022) 'new GSK' is the bet that vaccines (Shingrix GBP 3.6B, Arexvy), oncology (Blenrep relaunch, Jemperli) and long-acting-injectable HIV (Cabenuva) grow the company through the cliff. IMPORTANT DATA CAVEAT: yfinance reports GSK's financials in GBP but its EV and market cap in USD, so the headline EV/EBITDA (10.9x) is inflated by ~1.35x FX and every EV/EBITDA-derived fair value here (the $50 blend, the sensitivity grid) is biased low; the forward-P/E lens (~$57-61 at peer parity) is the reliable read, and the EV/EBIT sum-of-parts ($102) is a peer-parity ceiling, not a target. Simulated forward, the honest anchor is the driftless option-implied median of ~$49 (spot), not the drift-inflated bootstrap median of $61.**

- **Genuinely cheap on the one clean lens, and a real value-creator.** GSK trades at 10.1x forward earnings versus a 14.6x large-cap-pharma peer median (a 31% discount) and 16.4x trailing versus 23.6x (-30%), while earning a 22.2% ROIC against a 7.5% cost of capital — a +14.7-point spread that ties Merck (22.2%) for the highest in the peer set and towers over Pfizer (6.1%) and Sanofi (6.7%). This is not a broken business trading cheap; it is a high-return business the market refuses to re-rate.
- **The discount is the patent cliff, stated as math.** Per Mauboussin, GSK's 22.2% ROIC warrants a 19x forward P/E at just 3% sustainable growth; the market pays 10.1x, which implies roughly -4.4% durable growth. The market is explicitly pricing decline, and the reason is dolutegravir: the ViiV HIV oral franchise (HIV GBP 7.7B, GSK's largest single earner) faces loss of exclusivity around 2028-29. Everything bullish about GSK is a claim that vaccines, oncology and long-acting-injectable HIV grow the company through that cliff.
- **A currency artifact is hiding value — read forward P/E, not EV/EBITDA.** yfinance carries GSK's financials in GBP but its EV and market cap in USD (GBPUSD 1.3482), so the headline EV/EBITDA of 10.9x is inflated by ~1.35x FX; the currency-consistent figure is ~8x. Every EV/EBITDA-derived number here — the $50 segment blend, the $52 sensitivity base — is biased LOW for the same reason and sits artificially near spot. The forward-P/E lens implies ~$57-61 at peer parity, and the EV/EBIT sum-of-parts ($102) is what full re-rating to US-pharma multiples on properly-converted profit would look like: a ceiling that measures the embedded discount, not a price target.
- **Post-Haleon 'new GSK' is a vaccines-plus-specialty growth story.** Since demerging its consumer-health arm as Haleon in July 2022 (the biggest European

listing in a decade), GSK is biopharma-only. FY2025 delivered GBP 32.67B of turnover (+7% CER) and 29.9% core margin (+110bps), led by Specialty Medicines GBP 13.5B (+17%; Oncology +43%, HIV +11%, Respiratory/Immunology +18%) and Vaccines GBP 9.2B (Shingrix GBP 3.6B, +8%). The growth mix is richening; the question is whether it out-runs the cliff.

- **Both defining events of the modern GSK are visible in the price history.** The largest single-day up-move in the window, +8.7% on 19-Jul-2022, is the Haleon demerger reprice; the Zantac litigation then whipsawed the stock — a -6.7% panic on 11-Aug-2022 (the ~GBP 30B sector value wipe-out), a +7.8% relief on 6-Dec-2022 (the federal MDL tossed ~50,000 claims), and a -8.7% setback on 3-Jun-2024 (a Delaware ruling let ~70,000 state cases proceed). The overhang is now largely resolved: a ~$2.2B settlement covers ~93% of US state cases.
- **Balance sheet is adequate; free cash flow is the soft spot.** GSK carries modestly more leverage than peers (net debt/EBITDA 1.4x vs 1.2x, D/E 1.08 vs 0.76) but better interest coverage (11.8x vs 9.8x) and a lower, well-covered payout (69% vs 97.5%). The genuine weakness is cash conversion — a 14.6% FCF margin and 83% FCF conversion both trail peers, which is exactly why GSK screens 91% 'expensive' on price/FCF even while looking cheap on earnings.
- **Simulated forward, tilt up but anchor on the driftless null.** The 12-month bootstrap Monte Carlo median is $61 (75% chance above spot), but that embeds the recovery drift off the $35 low; the driftless, option-implied cone (IV 29.9%, low for the market) puts the 12-month median at ~$49 with a 44% chance above spot — i.e. roughly spot. The scenario grid shows resilience either way: even a bear -6% revenue case keeps core operating profit at GBP 8.7B and the dividend covered.

Confidence: MODERATE. The segment and product data are well-sourced (GSK FY2025 results, 4-Feb-2026; Q1 2026 pipeline appendix), the ROIC/peer/technical figures are clean, and the qualitative competitive and litigation facts were verified as of Aug 2026. Three things cap confidence. First, a genuine data artifact: GSK reports in GBP while the ADR prices in USD, so yfinance's EV/EBITDA and every multiple built on it are FX-distorted — this report leans on forward P/E and flags the rest. Second, GSK does not disclose clean core operating profit by commercial segment, so the sum-of-parts profit inputs are estimates and the SOTP is indicative only. Third, the central variable — the depth and timing of the dolutegravir/HIV cliff and the offsetting launch ramp — is a multi-year clinical and commercial question no model here forecasts. Competitive and legal claims are perishable.

WORKING PAPER

## 1 · Sector & industry trends

Large-cap pharma in 2026 is a two-speed sector, and GSK sits on the slower track for a specific reason. The dominant sector force is the patent-cliff supercycle: a cluster of the industry's biggest

earners — GSK's dolutegravir, Merck's Keytruda, Bristol's Eliquis, and others — lose exclusivity across 2026-2030, so the entire group is being valued on the credibility of its pipeline replacement rather than its trailing earnings. On top of that sit three cross-currents. First, US drug-pricing policy: the Inflation Reduction Act's Medicare negotiation and 'most-favoured-nation' pricing pressure, plus the on-and-off threat of pharmaceutical import tariffs (which took the whole group down in the April-2025 selloff), compress the US margin pool that funds R&D. Second, a genuine innovation cycle in vaccines (RSV, next-generation shingles and meningitis), oncology (ADCs, immuno-oncology combinations) and obesity — though GSK, notably, has no GLP-1 franchise, which is both a missed tailwind and a reason it is cheap. Third, a defensive-rotation and rate-cut backdrop that flatters cash-generative, high-dividend-yield compounders, of which GSK (3.5% yield) is one. The measurable sector fact in GSK's own data is a weekly ADF p-value of 0.744 — the price is statistically a near-random walk, which is what a large, diversified, defensive pharma should look like. The leaders in this sector are the pipeline-and-launch stories; GSK is a patent-cliff-defense story, and the two are priced very differently.

| Trend | Direction | Read for GSK |
|---|---|---|
| The industry-wide patent cliff (2026-2030) | ▼ Headwind | The dominant driver. GSK's dolutegravir (HIV GBP 7.7B, its largest earner) loses exclusivity ~2028-29; the group is valued on pipeline replacement, which is why GSK's 22.2% ROIC earns only a 10.1x forward multiple implying -4.4% durable growth. |
| US drug-pricing policy — IRA negotiation, MFN, tariffs | ▼ Headwind | Compresses the US margin pool that funds R&D. Pharma tariffs took the whole group down in April 2025 (GSK -6.4% on 4-Apr-2025). A standing overhang on the sector's highest-margin geography. |
| Vaccine innovation cycle — RSV, shingles, meningitis | ▲ Tailwind | GSK is a vaccines leader: Shingrix GBP 3.6B, Arexvy in RSV, a first pentavalent meningitis vaccine and a maternal-RSV Phase III. Vaccines GBP 9.2B is ~28% of sales and a genuine growth and moat asset. |
| RSV competitive dynamics (Pfizer, Moderna) | ◆ Neutral | Arexvy (GBP 0.6B) leads on efficacy but the US ACIP narrowed the older-adult recommendation, capping the class; Pfizer's Abrysvo and Moderna's mRESVIA split a smaller-than-hoped market. Upside is age-band and geographic expansion plus maternal RSV. |
| Long-acting-injectable HIV — the cliff defense | ▲ Tailwind | ViiV's Cabenuva and Apretude (long-acting) are growing 40-75% per quarter and shift the franchise from daily orals (which face the 2028-29 cliff) to differentiated injectables with patents into the 2030s, plus an ultra-long-acting (twice-yearly) pipeline. |
| No GLP-1/obesity franchise | ▼ Headwind | GSK is absent from the sector's single largest growth pool. This is a reason it is cheap versus peers with obesity exposure, and a strategic gap the pipeline does not fill. |
| Oncology build-out (ADCs, immuno-oncology) | ▲ Tailwind | Oncology grew +43% to GBP 2.0B; the Blenrep myeloma relaunch (approved 2025) is guided as a key 3-4-year growth driver and Jemperli is expanding in endometrial and colorectal cancer — the |

| | | |
|---|---|---|
| oncology) | | newest leg of the growth story. |
| Zantac (ranitidine) litigation resolution | ▲ Tailwind | The ~GBP 30B overhang of 2022 is largely resolved: a ~$2.2B settlement covers ~93% of US state cases and the federal MDL was dismissed (on appeal). Residual, but no longer existential. |
| Defensive rotation + rate cuts | ▲ Tailwind | A 3.5% yield, 25.5% realized volatility (low) and near-random-walk price make GSK a classic defensive compounder that benefits when capital rotates toward cash-generative, high-yield names. |
| UK/FTSE listing discount | ▼ Headwind | UK-listed large-caps trade at a persistent discount to US peers; part of the gap between GSK's 10.1x forward P/E and the US-pharma cohort is domicile, not fundamentals, and the USD-ADR/GBP-accounts mismatch compounds the optical cheapness. |

*Table 1 — the trends shaping GSK's markets, and what each means for the company.*

## 2 · What is driving GSK

GSK's price is driven, in descending order, by the HIV patent-cliff narrative, the vaccines franchise, the oncology build-out, and the fading Zantac overhang. On the **HIV cliff**, the tension is that ViiV is simultaneously GSK's biggest strength (HIV GBP 7.7B, +11%, the highest-return part of the company) and its biggest forward risk (dolutegravir loses exclusivity ~2028-29). The defense is explicit and already working: long-acting injectables Cabenuva and Apretude are growing 40-75% per quarter, moving revenue from vulnerable daily orals to differentiated injectables, and the pipeline carries ultra-long-acting (twice-yearly) assets and a broadly-neutralising antibody behind them. On **vaccines**, Shingrix (GBP 3.6B, +8%) is a durable franchise expanding into new geographies (a strong France launch), Arexvy is the efficacy leader in a class the US ACIP capped, and GSK is pushing a first pentavalent meningitis vaccine and a maternal-RSV Phase III — vaccines are ~28% of sales and the clearest growth-and-moat asset. On **oncology**, the +43% growth to GBP 2.0B, the Blenrep myeloma relaunch (guided a key 3-4-year driver after its 2022 US withdrawal), and Jemperli's expansion are the newest and fastest-growing leg. On **Zantac**, the ~$2.2B settlement covering ~93% of US state cases has moved the litigation from existential (the 2022 ~GBP 30B fear) to a manageable residual. Overlaying all of it: the post-Haleon 'new GSK' identity — biopharma-only since July 2022 — a 29.9% core margin (+110bps), a well-covered 3.5% dividend, and management's reaffirmed medium-term outlook. The re-rating catalyst is not a single event; it is quarters of evidence that Specialty and Vaccines out-grow the HIV cliff.

## 2b · Leading-indicator tracker

The tracker is weighted toward the segment mix, the HIV-franchise conversion and the two overhangs (Zantac, the ViiV minority). Note the tension: GSK's *operations* are performing well (FY2025 turnover +7% CER, core margin +110bps, Specialty +17%, Oncology +43%, HIV long-acting injectables +40-75%/quarter) while the *valuation* stays depressed on the forward cliff. That gap — strong current execution against a discounted forward — is the entire investment debate for this name.

debate for this name.

| Indicator | Latest | Prior | Read | Source |
|---|---|---|---|---|
| FY2025 total turnover | GBP 32.67B (+4% AER / +7% CER) | GBP ~31.4B (FY2024) | Above the mid-single-digit algorithm; Specialty carried the growth | GSK FY2025 |
| FY2025 core operating margin | 29.9% (+110bps) | ~28.8% | Margin expanding as Specialty mix richens | GSK FY2025 |
| Specialty Medicines sales | GBP 13.5B (+17%) | GBP ~11.5B | HIV GBP 7.7B (+11%), Oncology GBP 2.0B (+43%), R&I GBP 3.8B (+18%) - the growth engine | GSK FY2025 |
| Vaccines sales | GBP 9.2B (+2%) | GBP ~9.0B | Shingrix GBP 3.6B (+8%), Meningitis GBP 1.6B (+12%); Arexvy GBP 0.6B still depressed by narrowed US RSV recommendation | GSK FY2025 |
| General Medicines sales | GBP 10.0B (-1%) | GBP ~10.1B | Trelegy GBP 3.0B (+13%) offsetting established-product erosion | GSK FY2025 |
| HIV franchise (ViiV) | GBP 7.7B (+11%) | GBP ~6.9B | Long-acting injectables (Cabenuva, Apretude) growing 40-75%/qtr - the defense against the oral-dolutegravir cliff | GSK 2025 |
| Zantac (ranitidine) litigation | ~$2.2B settlement resolves ~93% of US state cases; ~2,233 federal MDL cases dismissed (plaintiffs appealing); $70M Valisure qui tam agreed | ~GBP 30B market-cap wipe-out fear (2022) | Overhang largely resolved through 2024-2025; residual federal appeal + non-participating cases remain | GSK / pharmaphorum / classaction.org 2024-2025 |
| ViiV shareholding | Pfizer exited (sold 11.7% for $1.875B); Shionogi 10% -> | GSK 78.3% / Pfizer 11.7% / | Simplifies the minority structure; GSK | GSK/Shionogi |

| | | | | |
|---|---|---|---|---|
| change | 21.7%; GSK holds 78.3% | Shionogi 10% | ownership unchanged at 78.3% | Jan 2026 |
| Blenrep (belantamab) relaunch | Approved (combinations) in US/EU/UK/Japan 2025 for R/R multiple myeloma | Withdrawn from US 2022 (failed confirmatory trial) | GSK guides Blenrep a key growth driver next 3-4 years - oncology re-entry | GSK 2025 |
| Bepirovirsen (hep B) Phase III | B-Well 1 & 2 met primary endpoint (functional cure); filings from Q1 2026 | Phase IIb | Potential first functional-cure therapy for chronic HBV - a pipeline highlight | GSK Jan 2026 |
| Blujepa (gepotidacin) launch | Approved uncomplicated UTI + urogenital gonorrhoea | n/a | First-in-class oral antibiotic - novel mechanism, modest near-term sales | GSK 2025 |

*Table 2 — operating KPIs with latest readings. GSK FY2025 results (4-Feb-2026) and Q1 2026 results; company pipeline & clinical-trials appendix (Q1 2026); Zantac litigation updates (GSK statements, pharmaphorum, classaction.org, 2024-2025); ViiV shareholding change (GSK/Shionogi, Jan 2026). Verified as of Aug 2026 and perishable.*

## 2c · Pipeline & key products

GSK's revenue is unusually well-diversified across four distinct engines — HIV (ViiV), vaccines, respiratory/immunology and a growing oncology book — which is the structural answer to the patent-cliff question: no single product is more than ~11% of group sales, and the biggest concentration (the HIV franchise) is being actively converted from orals to long-acting injectables. The table below lists the top marketed products with their latest full-year sales and exclusivity horizon. Two things stand out: the loss-of-exclusivity risk is concentrated in the 2027-2030 window (dolutegravir orals ~2028-29, Trelegy ~2029-30, Nucala ~2027, Benlysta ~2026), and the growth offset is concentrated in vaccines (Shingrix, Arexvy, meningitis) and the newer specialty/oncology launches. Figures are GBP full-year 2025; segment sub-totals are exact from GSK's results, individual sub-brand figures marked '~' are approximate.

| Product | Therapy area | Latest FY sales | Exclusivity / LOE |
|---|---|---|---|
| HIV franchise / ViiV (Dovato, Tivicay, Cabenuva, Apretude, Juluca) | Specialty — HIV | GBP 7.7B (+11%) | Oral dolutegravir ~2028-29 (US/EU); long-acting injectables patented into 2030s |
| Shingrix | Vaccines — shingles | GBP 3.6B (+8%) | No near-term cliff; patents ~2032; competition risk, not LOE |

| | | | |
|---|---|---|---|
| Trelegy Ellipta | General Medicines — COPD/asthma | GBP 3.0B (+13%) | US device/formulation LOE ~2029-30 |
| Meningitis (Bexsero / Menveo) | Vaccines — meningitis | GBP 1.6B (+12%) | Franchise; pentavalent MenABCWY next-gen extends runway |
| Nucala (mepolizumab) | Specialty — Respiratory/Immunology | ~GBP 1.7B (within R&I GBP 3.8B) | US LOE ~2027; depemokimab is the successor |
| Benlysta (belimumab) | Specialty — Immunology (lupus) | ~GBP 1.5B (within R&I GBP 3.8B) | Biologic; US LOE ~2026, biosimilar entry gradual |
| Arexvy | Vaccines — RSV (60+) | GBP 0.6B (+2%) | Patents ~2035; near-term risk is competition (Pfizer, Moderna) + ACIP scope, not LOE |
| Zejula (niraparib) | Oncology — ovarian (PARP) | ~GBP 0.5B (declining) | LOE ~2026-28; US sales already eroding |
| Jemperli (dostarlimab) | Oncology — endometrial (PD-1) | Growing (within Oncology GBP 2.0B, +43%) | Biologic; long runway ~2033+; label expansions ongoing |

*Table 3 — top marketed products: therapy area, latest full-year sales and exclusivity/loss-of-exclusivity (LOE) horizon. Source: GSK FY2025 results (4-Feb-2026) product-sales tables; segment sub-totals are exact, sub-brand figures marked '~' are approximate; LOE horizons are analyst estimates from patent-expiry disclosures and are indicative.*

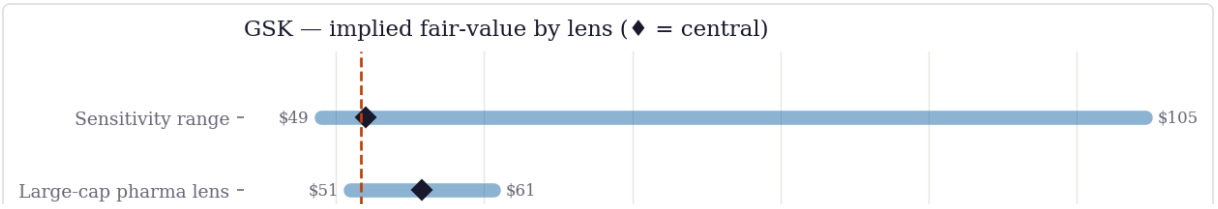
The breadth of the pipeline is the point — this is the 'variety of scripts' that underwrites the cliff-defense thesis. GSK is running late-stage programmes across all four of its engines simultaneously: respiratory/immunology (the ultra-long-acting anti-IL5 depemokimab, six-monthly dosing), HIV (ultra-long-acting and antibody-based regimens to succeed dolutegravir), oncology (Blenrep's relaunch, Jemperli and Zejula expansions) and vaccines (maternal RSV, pentavalent meningitis, next-generation flu), plus a genuinely novel infectious-disease cluster (the antisense hepatitis-B 'functional cure' bepirovirsen, the first-in-class oral antibiotic gepotidacin/Blujepa, and oral carbapenem tebipenem). The table shows the spread across phase and therapy area. The strategic read: the depth in vaccines and specialty is real and diversified, but none of these individually replaces dolutegravir's scale — the thesis is that the aggregate of many mid-sized launches, plus the LAI conversion of HIV itself, out-runs the cliff.

| Asset | Therapy area | Phase / status | Note |
|---|---|---|---|
| Depemokimab (Exdensur) | Respiratory / Immunology | Approved (asthma, CRSwNP); Ph III EGPA (H2 2026), COPD | Ultra-long-acting anti-IL5, 6-monthly dosing; Nucala successor |
| Bepirovirsen | Infectious disease — chronic | Ph III positive (B-Well 1 & | Antisense; potential first functional cure for chronic |

| | | | |
|---|---|---|---|
| Bepirovirsen | — chronic hepatitis B | 2); filings from Q1 2026 | functional cure for chronic HBV |
| Blenrep (belantamab mafodotin) | Oncology — multiple myeloma | Approved 2025 (combinations), relaunch | ADC; guided a key growth driver for 3-4 years after 2022 US withdrawal |
| Blujepa (gepotidacin) | Infectious disease — UTI / gonorrhoea | Approved | First-in-class oral antibiotic (novel mechanism); modest near-term sales |
| Jemperli (dostarlimab) | Oncology — endometrial + colorectal | Approved + Ph III expansions | PD-1; expanding beyond endometrial into earlier lines and new tumours |
| Camlipixant | Respiratory — refractory chronic cough | Phase III | P2X3 antagonist (via Bellus acquisition); large unmet-need indication |
| Long-acting / antibody HIV (cabotegravir next-gen, N6LS bnAb) | HIV | Phase II/III | Ultra-long-acting (twice-yearly) regimens — the dolutegravir-cliff defense |
| Maternal RSV vaccine | Vaccines — RSV (newborns) | Phase III | Protects newborns up to ~6 months via maternal immunisation |
| Pentavalent meningitis (MenABCWY) | Vaccines — meningitis | Filed / late-stage | First pentavalent meningitis vaccine; consolidates the GBP 1.6B franchise |
| Tebipenem (oral carbapenem) | Infectious disease — cUTI | Phase III / filed | Oral carbapenem (via Spero); hospital-to-home antibiotic |
| Next-generation flu + gonorrhoea vaccines | Vaccines | Phase I/II | Earlier-stage breadth across the vaccines franchise |

*Table 4 — pipeline by phase & therapy area, showing the breadth across respiratory/immunology, HIV, oncology and vaccines. Source: GSK Q1 2026 pipeline assets and clinical-trials appendix and FY2025 results (4-Feb-2026); phases as disclosed. Clinical outcomes are uncertain and dates are indicative.*

## 3 · Valuation — where GSK should trade



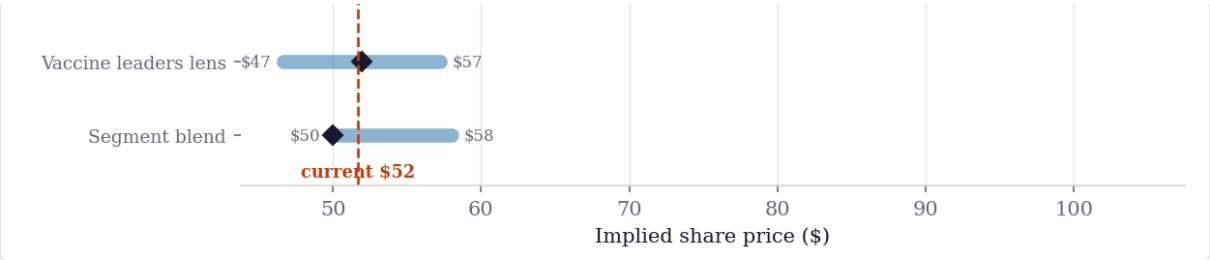



**Fig. 1.** Implied fair-value by lens; dashed line = current $52.

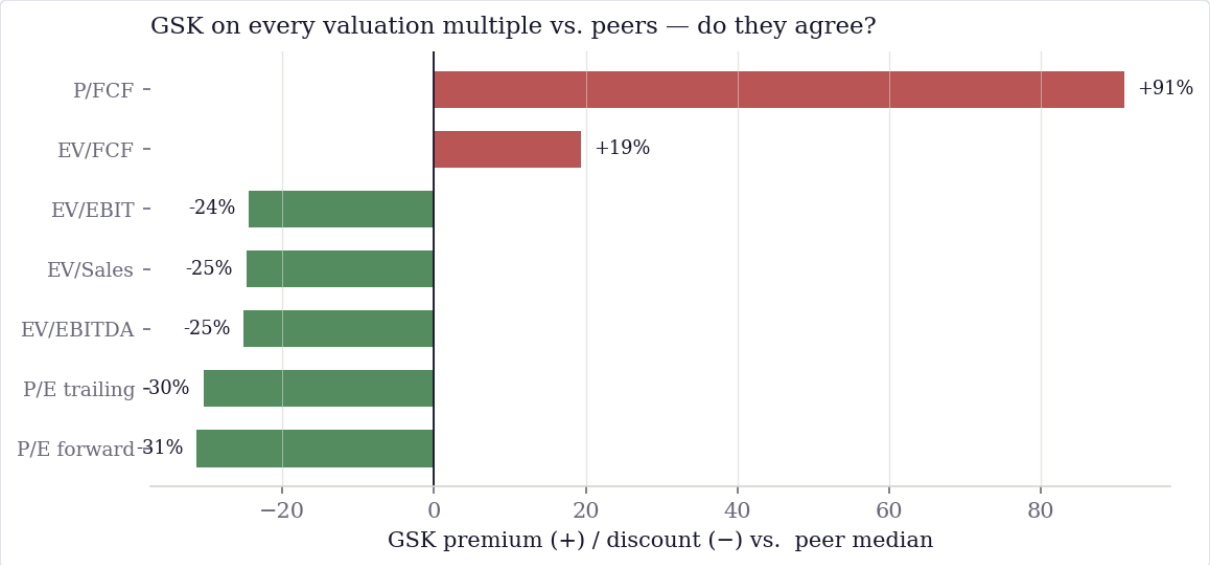



**Fig. 2.** GSK premium/discount to its subsector peers by valuation multiple.

| Metric | GSK | Peer median | Read |
|---|---|---|---|
| P/E trailing | 16.41 | 23.59 | **-30% cheap** |
| P/E forward | 10.06 | 14.65 | **-31% cheap** |
| EV/EBITDA | 10.93 | 14.60 | **-25% cheap** |
| EV/EBIT | 14.62 | 19.35 | **-24% cheap** |
| EV/Sales | 3.56 | 4.74 | **-25% cheap** |
| P/FCF | 29.74 | 15.57 | **+91% expensive** |
| EV/FCF | 33.96 | 28.45 | **+19% expensive** |
| Dividend yield | 3.52 | 3.04 | **+16% expensive** |

*Table 5 — GSK on every multiple vs. peer median.*

| Lens | EV/EBITDA | Fwd P/E | Implied $ (EV/EBITDA) | Implied $ (Fwd P/E) |
|---|---|---|---|---|
| Large-cap pharma | +1% | -15% | $51 | $61 |
| Vaccine leaders | +9% | -10% | $47 | $57 |

*Table 6 — segment-relevant peer lenses, premium/discount and implied prices.*

Segment-weighted blend: **revenue-weighted $50**, **profit-weighted $50**.

## 3a · Sensitivity

| EBITDA ↓ / mult → | 10.9× | 16.7× | 19.5× | 22.3× |
|---|---|---|---|---|
| **$6.7B** | $29 | $49 | $58 | $67 |
| **$8.5B** | $39 | $64 | $75 | $87 |
| **$10.4B** | $49 | $79 | $94 | $108 |
| **$10.8B** | $51 | $83 | $98 | $113 |
| **$11.4B** | $55 | $88 | $104 | $120 |

*Table 7 — implied price = (EV/EBITDA × EBITDA − net debt) ÷ shares. Base $52.*

## 3b · Return on invested capital

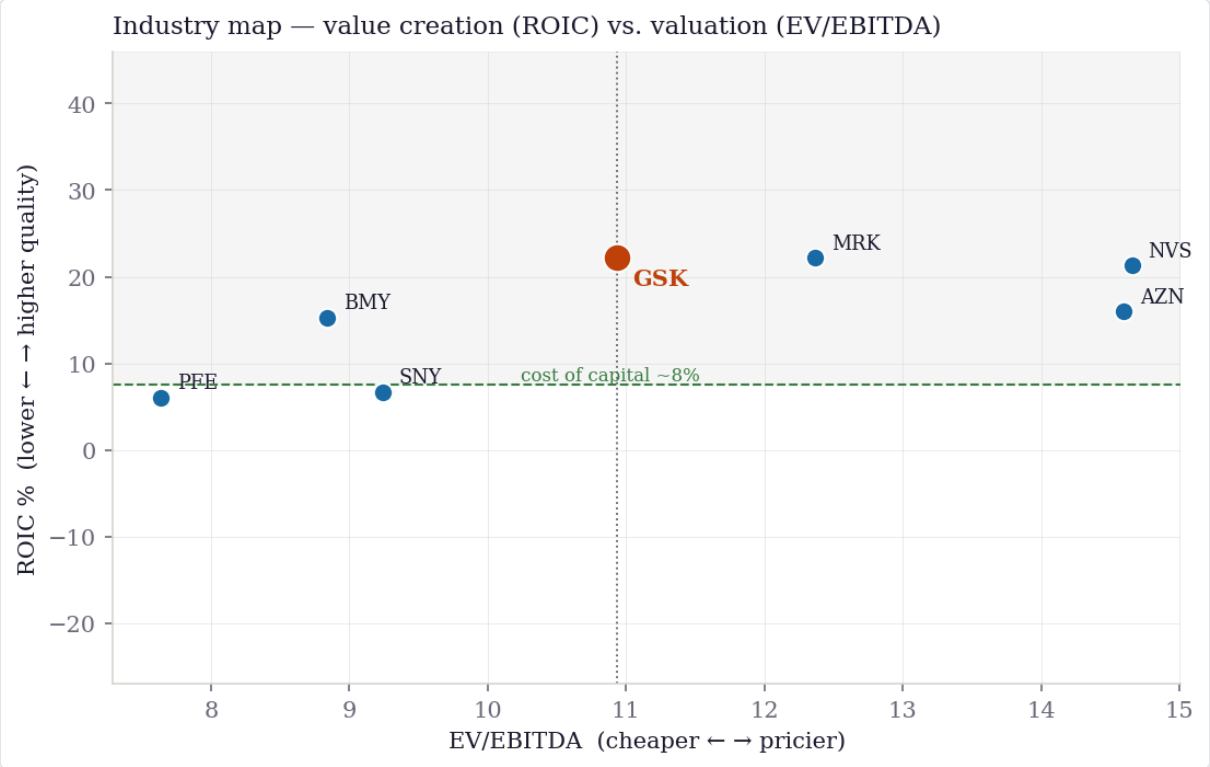



**Fig. 3.** Value creation (ROIC) vs. valuation (EV/EBITDA). GSK highlighted.

GSK's ROIC is **22.2%** vs. an assumed cost of capital of ~8% — above (value-creating). Per Mauboussin (Morgan Stanley, *Valuation Multiples*), the warranted multiple is a function of the ROIC-vs-WACC spread and growth; a low-return business should trade at a modest multiple until

returns improve.

**Lease-adjusted ROIC bridge.** Leases are immaterial for GSK (a ~GBP 0.8B lease obligation against a large invested-capital base), so the three ROIC treatments sit almost on top of each other — 22.8% ex-lease, 22.3% lease-adjusted, 22.2% pipeline headline — and the lease adjustment carries no information for this name; it is retained only for cross-report comparability. The figure that matters is the level: a ~22% ROIC on GBP 8.1B of operating profit is genuine value creation, well above the 7.5% cost of capital, and it is the fact that makes GSK's cheapness interesting rather than a value trap. The caveat is not leases but *consolidation*: this ROIC is struck on consolidated NOPAT that includes 100% of ViiV, of which GSK owns 78.3% — the economic-attributable return is marginally lower, though ViiV's high margins mean the direction, not the magnitude, is what changes.

| Basis | Invested capital | ROIC |
|---|---|---|
| Ex-lease (financial capital only) | $30.12B | 22.8% |
| Lease-adjusted (capitalize leases + interest add-back) | $30.95B | 22.3% |
| Pipeline headline (lease in denominator only) | $30.95B | 22.2% |

*Table 8 — ROIC under three invested-capital treatments. Ex-lease (financial-capital) ROIC ignores capitalized lease obligations and is flattered; the lease-adjusted ROIC capitalizes the ~$0.8B lease liability into invested capital and adds the implied lease interest back to NOPAT (the economically correct read); the pipeline headline puts the lease in the denominator only (slightly conservative). Spread vs. the ~7.5% cost of capital is shown in the table.*

**Mauboussin warranted multiple.** Per Mauboussin & Callahan (*Valuation Multiples*, Morgan Stanley), a warranted multiple is a function of the ROIC-minus-WACC spread and growth. GSK's 22.2% ROIC against a 7.5% WACC is a wide +14.7-point spread that mechanically warrants a **19x forward P/E at just 3% sustainable growth**, rising to 23x at 4% and 31x at 5%. The market pays **10.1x**. Solving for the growth that reconciles the actual multiple gives roughly **-4.4% implied durable growth**: the market is not pricing GSK as a low-return business (it is high-return) — it is pricing it as a *declining* one. That is the patent cliff, stated as arithmetic. Unlike the Barrick case (where the framework broke because a cyclical miner has no steady-state ROIC), the framework is *informative* here: GSK genuinely earns a durable, high ROIC, so the entire gap between the 19x warranted and 10.1x actual is a bet on whether post-cliff growth is negative (the market's view) or merely low-but-positive (the bull's view). Every point of durable growth GSK can prove is worth several turns of multiple. This is the cleanest single frame for the whole thesis: the re-rating is a growth-credibility question, not a quality question.

| Sustainable growth g | Warranted fwd P/E |
|---|---|
| 3.0% | 19.2× |
| 4.0% | 23.4× |
| 5.0% | 31.0× |
| 6.0% | 48.6× |

| | |
|---|---|
| 7.0% | 136.9× |

*Table 9 — warranted forward P/E = (1 − g/ROIC)/(WACC − g) at ROIC 22.2% and WACC 7.5%. Actual forward P/E 10.1× ≈ -4.4% implied durable growth.*

## 3d · Segment sum-of-parts (reported 10-K segment profit)

The sum-of-parts values each commercial segment's *estimated* operating profit — Specialty (struck on an attributable, ex-ViiV-minority basis), Vaccines and General Medicines — at its lens median EV/EBIT multiple, converted to USD (at GBPUSD 1.3482) so the arithmetic is internally consistent with the engine's USD net debt and USD share price. It lands at **$102**, roughly double spot. **That number must be read carefully, and it is not a price target.** It applies the large-cap-pharma lens's EV/EBIT median of ~18x to GSK's operating profit — i.e. it assumes GSK closes its *entire* valuation discount to US pharma peers. GSK actually trades at roughly half that EV/EBIT multiple on a currency-consistent basis, and the market withholds the re-rating for concrete reasons: the 2028-29 HIV cliff, weaker FCF conversion, no obesity franchise, and the UK domicile discount. Read the $102 as a *ceiling that measures the size of the embedded discount*, not as fair value. Two methodological points are material. First, following the Barrick/Nevada-Gold-Mines lesson, ViiV is consolidated but only 78.3% owned (Shionogi holds 21.7% after Pfizer sold its stake to Shionogi in early 2026), so Specialty's profit is struck net of Shionogi's ~21.7% share of estimated HIV operating profit rather than valuing 100% of ViiV as attributable. Second, GSK does not disclose clean segment operating profit, so the segment profit inputs are *estimates* from disclosed group margin and typical contribution margins — the SOTP is indicative, and the currency-clean forward-P/E lens blend (~$57-61 at peer parity) is the better-grounded anchor.

| Segment | Lens | Seg. profit | EV/EBIT | Segment EV | Implied $/sh |
|---|---|---|---|---|---|
| Specialty Medicines (attributable, ex-ViiV minority) | Large-cap pharma | $6.16B | 18.26× | $112.48B | $56.0 |
| Vaccines | Vaccine leaders | $4.31B | 17.68× | $76.2B | $38.0 |
| General Medicines | Large-cap pharma | $2.97B | 18.26× | $54.23B | $27.0 |
| **Gross segment EV** | | | | **$242.91B** | |
| less: corporate/unallocated (× 18.07×) | | | | −$24.4B | |
| less: net debt (lease-inclusive) | | | | −$14.72B | |
| **Equity value → implied price** | | | | **$203.79B** | **$102.0** |

*Table 10 — each reported segment valued on its lens EV/EBIT multiple. Each reported segment's FY2026 segment profit (~operating EBIT) is valued at the median EV/EBIT of its lens peers (yfinance); unallocated general corporate expense is*

*capitalized at the profit-weighted blended multiple and subtracted; lease-inclusive net debt is deducted (peer EV and target net debt are both ASC-842 lease-inclusive, so the bases are consistent).*

SOTP implied **$102.0** vs the §3 EV/EBITDA blend ($50 rev / $50 profit) and spot $52 — the sum-of-parts sits above the blend. Indicative only. The SOTP stacks several estimates (segment operating profit is not disclosed; the ViiV attributable adjustment is approximate) and inherits the USD/GBP peer-multiple noise for the EUR/GBP reporters (SNY, NVS) in the lens. Read it as a cross-check on the forward-P/E lens blend, not as a precise target.

**Source.** GSK FY2025 results (4-Feb-2026): segment turnover Specialty GBP 13.5B / Vaccines GBP 9.2B / General GBP 10.0B; group core operating profit GBP 9.78B at 29.9% margin. SEGMENT OPERATING PROFIT IS ESTIMATED (GSK does not disclose clean core operating profit by commercial segment - central R&D and corporate are unallocated); segment profit is estimated from disclosed group margin and typical segment contribution margins and is labeled an estimate. Converted to USD at GBPUSD 1.3482 so the sum-of-parts is internally consistent with the engine's USD net debt and USD share price (a peer EV/EBIT multiple is currency-neutral, so applying it to USD-converted GSK EBIT yields a USD enterprise value that reconciles with USD net debt).

## 3e · Key-product / LOE scenarios -> revenue, core operating profit & free cash flow

This grid replaces the retail comp-and-margin construction with the two variables that actually move a biopharma's cash flow through a patent cliff: the net revenue trajectory (the HIV/dolutegravir loss of exclusivity netted against the vaccines and oncology launch ramp) and the core operating margin. It is calibrated to GSK's reported FY2025 base — GBP 32.67B turnover, 29.9% core margin, ~48% FCF conversion of core operating profit, a ~GBP 2.6B dividend. The finding is **resilience**: even the bear case (revenue -6%, a steep HIV cliff plus Zantac residual plus vaccine softness) holds core operating profit at GBP 8.7B, GBP 4.2B of FCF and GBP 1.6B retained after the dividend — the payout is covered in every scenario. The base (+3%) and bull (+7%, Shingrix/Arexvy plus oncology ramp plus a successful LAI defense of HIV) push core operating profit to GBP 10.2-11.0B. All figures are GBP to avoid the USD/GBP EV artifact, and every row is an analyst scenario, not company guidance.

| Scenario | Revenue Δ | Core op margin | Revenue | Core op profit | FCF | Retained after dividend |
|---|---|---|---|---|---|---|
| Bear - steep HIV cliff + Zantac residual + vaccine softness | -6% | 28.4% | GBP 30.7B | GBP 8.72B | GBP 4.19B | GBP 1.59B |
| Base - Specialty growth offsets HIV maturation; vaccines stable | +3% | 30.4% | GBP 33.7B | GBP 10.23B | GBP 4.91B | GBP 2.31B |
| Bull - Shingrix/Arexvy + oncology ramp; LAI | +7% | 31.4% | GBP | GBP | GBP | GBP 2.67B |

| | | | | | |
|---|---|---|---|---|---|
| defends HIV | | | 35.0B | 10.98B | 5.27B |

*Table 11 — Revenue = base x (1 + scenario revenue delta); core operating profit = revenue x (base margin + delta bps); FCF = core operating profit x base-year FCF conversion; retained = FCF - dividend. The revenue delta is dominated by the HIV/dolutegravir loss-of-exclusivity horizon (~2028-29) netted against the vaccines and oncology launch ramp. Analyst scenario grid - NOT company guidance.*

**Calibration.** base revenue b = 32.67 · base core margin pct = 29.9 · fcf conversion of core pct = 48.0 · dividend b = 2.6

**Source.** Base year = GSK FY2025 results (4-Feb-2026): turnover GBP 32.67B, core operating profit GBP 9.78B (29.9% margin), free cash flow ~GBP 4.76B (yfinance), full-year dividend ~GBP 2.6B (64p/ordinary share). Scenario revenue/margin deltas are analyst assumptions built around the dolutegravir/HIV loss-of-exclusivity horizon (~2028-29), the Shingrix/Arexvy/oncology growth ramp and Zantac residual risk - NOT company guidance. All figures in GBP to match GSK's reporting currency (avoids the USD/GBP EV artifact).

## 4 · Headwinds

**The dolutegravir/HIV loss of exclusivity is the dominant risk**, and it is what the whole discount prices: the ViiV oral franchise inside HIV's GBP 7.7B faces US/EU exclusivity loss around 2028-29, and while long-acting injectables (Cabenuva, Apretude) are converting the franchise, they do not yet fully replace the scale of the orals. **Free-cash-flow conversion is a genuine weakness** — a 14.6% FCF margin and 83% conversion both trail the peer medians (17.2% and ~100%), which is why GSK screens 91% expensive on price/FCF even as it looks cheap on earnings; the dividend is covered (69% payout) but the cash engine is thinner than the P&L suggests. **Zantac is resolved but not zero**: the ~$2.2B settlement covers ~93% of US state cases, yet the federal MDL dismissal is under appeal and non-participating claims and a Boehringer overhang remain. **US drug-pricing policy** — IRA negotiation, most-favoured-nation pressure and the recurring threat of pharma tariffs (which drove the -6.4% April-2025 move) — compresses GSK's highest-margin geography. **Arexvy/RSV is capped** by the narrowed US ACIP older-adult recommendation and a three-way competitive split with Pfizer and Moderna. **The absence of a GLP-1/obesity franchise** leaves GSK out of the sector's biggest growth pool. **The UK/FTSE domicile discount** and the USD-ADR/GBP-accounts mismatch keep the optical and real multiples below US peers. Finally, **the balance sheet carries modestly more leverage** than peers (net debt/EBITDA 1.4x, D/E 1.08) and **liquidity is tighter** (current ratio 0.82 vs 1.14) — not a solvency risk given 11.8x interest coverage, but less cushion than the group. Technically the setup is neutral, not a tailwind: price sits on its MA50 ($51.68) and MA200 ($51.05) with RSI 48.8 and %B 42 — consolidating, -14.1% off the 52-week high.

## 5 · Technical read

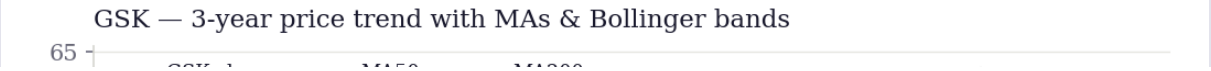





**Fig. 4.** 3-year price with MA50/200 & Bollinger bands.

| Indicator | Reading |
|---|---|
| MA50 / MA200 | $51.68 / $51.05 — golden cross |
| RSI(14) | 48.8 |
| Bollinger %B | 42% (upper $54.0) |
| 52-week range | $35.42–$60.14 (-14% off high) |
| Realized vol (90d, ann.) | 25.5% |

*Table 12 — technical dashboard.*

## 6 · Price forecast — weekly ETS, ARIMA & ADL (130-week window, 26-week horizon)

**Weekly re-basing.** Section 6 runs at weekly frequency over the trailing ~2.5 years (130 weeks) with a 26-week (~6-month) horizon, so all three models are estimated on the same window and are internally consistent. Weekly ADF p≈0.744 (near random walk); weekly return σ≈3.44%.

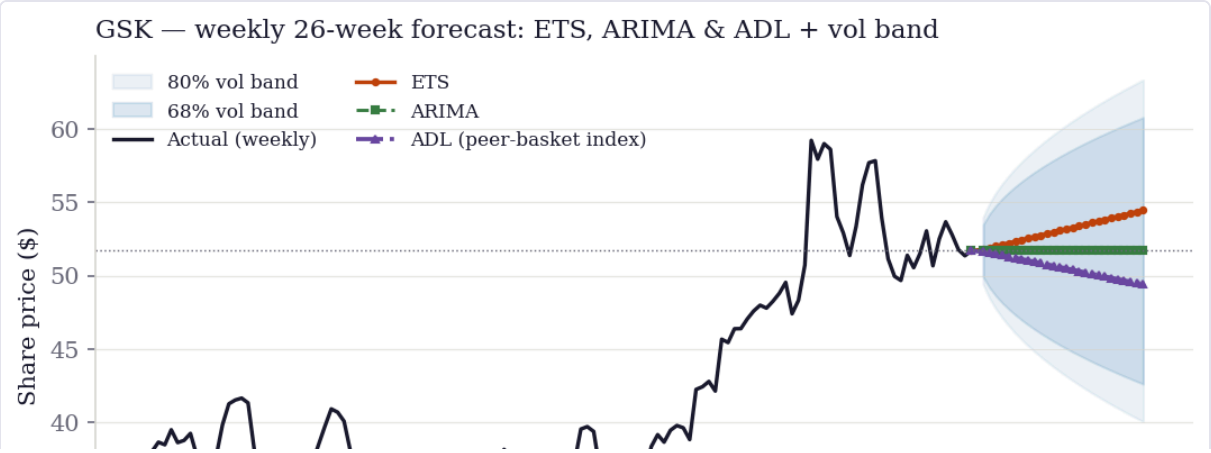





**Fig. 5.** Weekly 26-week forecast — ETS (trend), ARIMA (near-flat) and the ADL (peer-driven) — with a volatility-implied band.

| Horizon | ETS | ARIMA | ADL | 68% vol band |
|---|---|---|---|---|
| ~1 mo (4 wk) | $52 | $52 | $51 | $48–$55 |
| ~3 mo (13 wk) | $53 | $52 | $51 | $45–$58 |
| ~6 mo (26 wk) | $54 | $52 | $49 | $43–$61 |

*Table 13 — weekly point forecasts (ETS, ARIMA, ADL) and 68% volatility band. Scenario anchors, not predictions.*

## 6a · ADL model — a peer-driven forecast

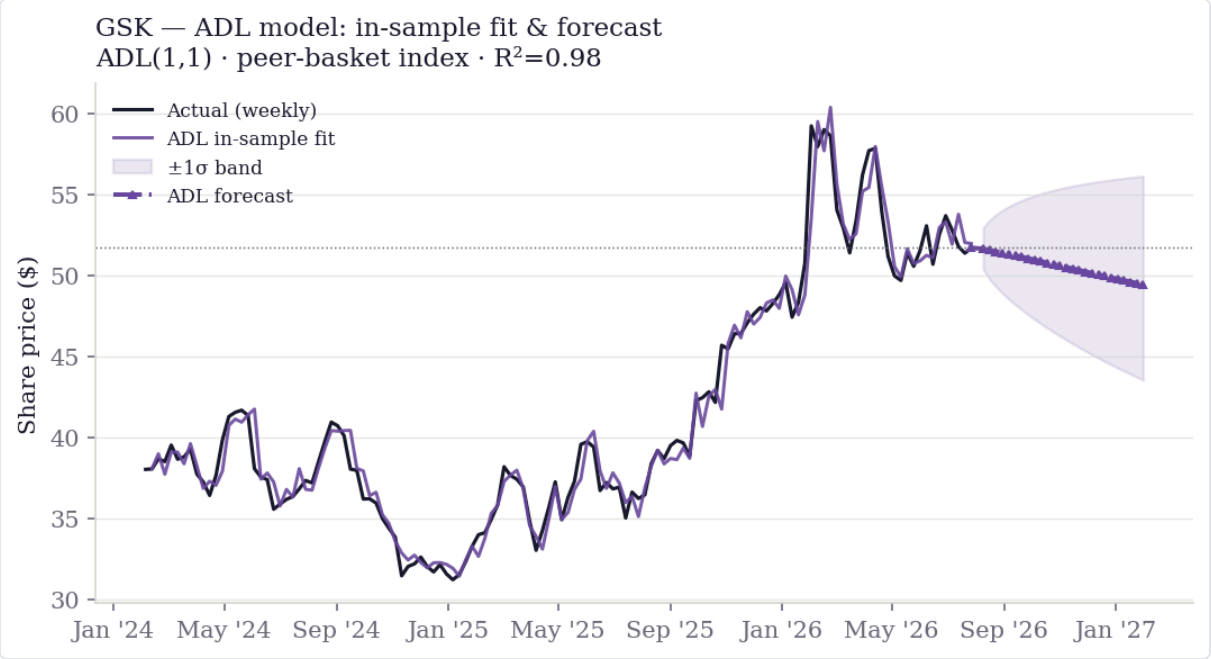



**Fig. 6.** ADL in-sample fit (purple) vs actual (black) and the 26-week forward path with a ±1σ band. The peer basket co-drives the price level.

**Specification.** ADL(1,1) — weekly log price regressed on 1 own lag(s) plus distributed lags (0..1) of the **peer-basket index**. Order chosen by AIC (grid search over exog sets (peer-basket index, SPY (broad market), peer-basket index + SPY, xlv basket (XLV), peer-basket index + xlv basket (XLV)) x p in 1..4 x q in 0..2, min AIC); AIC -579.3, BIC -567.9, in-sample R² 0.979. Peer basket (6 names): AZN, BMY, MRK, NVS, PFE, SNY. Multi-step exogenous inputs: each exogenous series projected forward by its own fitted AR(1) on log-levels. Interval is ±1σ (residual σ=0.0249 in log space) scaled by √horizon — approximate; it omits parameter and exogenous-projection uncertainty.

The Section 6 ADL was run with the Health Care sector ETF (XLV) added to the exogenous candidate grid alongside the default equal-weight large-cap-pharma peer basket and SPY, winner chosen by AIC over exog set x p in 1..4 x q in 0..2. The selected spec is **ADL(1,1) on the peer-basket index** (AIC -579.3, in-sample R2 0.979), and its 26-week path lands at **$49.42** under the AR(1) exogenous projection versus $53.02 with the peer basket held flat, against spot of $51.69 — both essentially at spot. **The AIC ranking is the real finding, and it is clean.** The pharma peer basket alone (-579.3) beats every alternative, including peer+SPY and peer+XLV (both -576.2, i.e. adding the ETFs slightly *worsens* AIC once the peer basket is present), and crucially **XLV alone (-542.8) beats SPY alone (-504.3)**: the healthcare sector explains GSK better than the broad market, but the narrow pharma-peer basket explains it better than either. This is the economically sensible ordering — GSK moves with its direct competitors first, its sector second, the market last — and unlike a self-referential holdings-ETF (the Barrick/GDX tautology), XLV only lightly overlaps GSK, so this is a genuine result. **The backtest reads well but should be discounted:** the rolling 1-step out-of-sample RMSE is $1.74 against $2.46 for a random walk (ratio 0.706), but the 1-step test feeds the model the realized contemporaneous peer-basket value, so it measures nowcasting conditional on knowing where the peers closed, not genuine forecasting. The honest forward summary is the ADF p-value of 0.744 and 3.4% weekly volatility: a near-random walk whose best anchor is spot.

## 7 · Monte Carlo & events

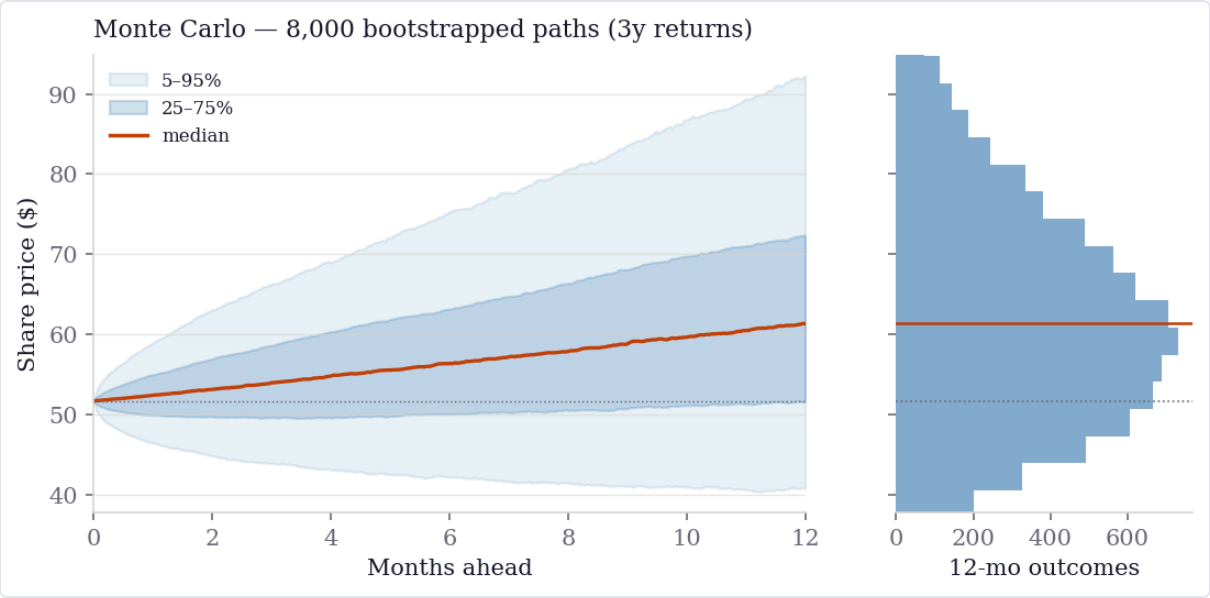



**Fig. 7.** 10k bootstrapped paths + 12-month distribution.

| Horizon | P5 | P25 | Median | P75 | P95 | P(>current) |
|---|---|---|---|---|---|---|
| 6mo | $42 | $50 | $56 | $63 | $75 | 69% |
| 12mo | $41 | $52 | $61 | $72 | $92 | 75% |

*Table 14 — Monte Carlo percentiles (embeds recent drift; driftless null ≈ current price).*

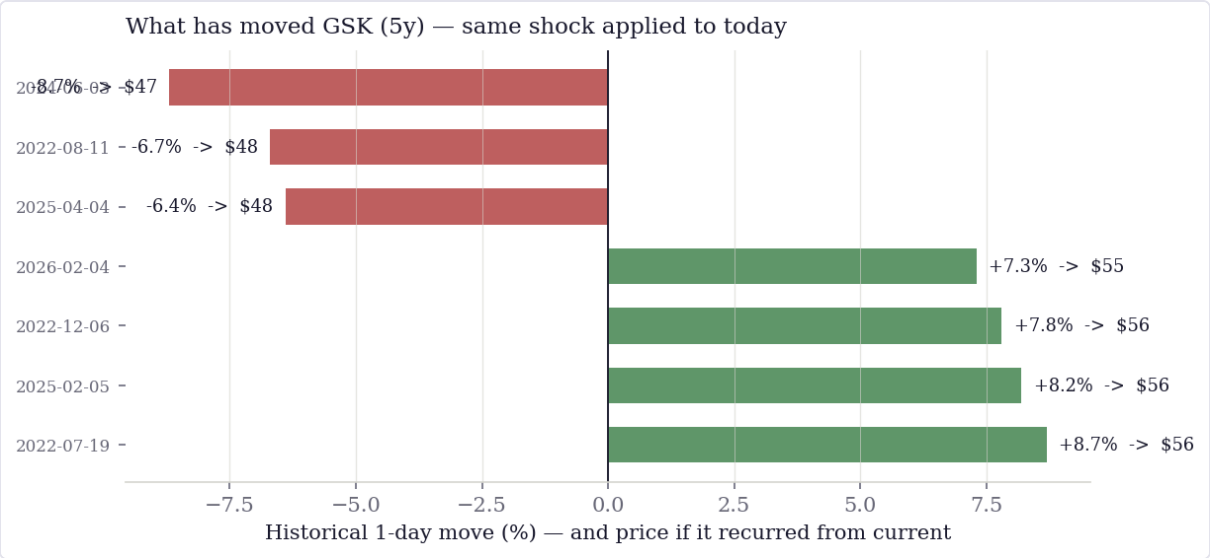



**Fig. 8.** GSK's largest single-day moves over the past 5 years and the price each would imply from today's $52.

| Date | 1-day move | Likely catalyst | If repeated from $52 |
|---|---|---|---|
| 2024-06-03 | **-8.7%** | Zantac setback: a Delaware Superior Court ruling permitted plaintiffs' expert testimony, allowing ~70,000 state-court cases to proceed to trial. GSK sold off sharply, -8.7% (the largest down-move in the window). | $47 |
| 2022-08-11 | **-6.7%** | Zantac (ranitidine) litigation panic: fears of NDMA/cancer liability wiped ~GBP 30B off the combined value of GSK, Haleon and Sanofi in early-to-mid August 2022 as US trial dates approached. The price shock that defined the overhang, -6.7%. | $48 |
| 2025-04-04 | **-6.4%** | Pharma-tariff selloff: the broad 'Liberation Day' tariff shock took the whole sector down; GSK moved with the group as pharma-import-tariff risk repriced, -6.4%. | $48 |
| 2026-02-04 | **+7.3%** | FY2025 results (confirmed results date): GBP 32.67B turnover (+7% CER), 29.9% core margin, reaffirmed medium-term outlook, +7.3%. | $55 |
| 2022-12-06 | **+7.8%** | Zantac relief: the US federal MDL judge (S.D. Florida) excluded plaintiffs' scientific expert testimony under Daubert, dismissing ~50,000 federal claims. A major litigation win, +7.8%. | $56 |
| 2025-02-05 | **+8.2%** | FY2024 results (confirmed results date): full-year print and outlook drove the move, +8.2%. | $56 |
| 2022-07-19 | **+8.7%** | Haleon demerger reprice: GSK separated its consumer-health business into Haleon plc on 18-Jul-2022 (the biggest European listing in a decade), leaving a biopharma-only GSK that repriced the next session. The structural event that created the modern company, | $56 |

+8.7%.

*Table 15 — the biggest GSK price events (5-year window) and their forward echo. The distinct-event history for GSK is unusually legible because it traces the two corporate events that made the modern company.* ***The single largest up-move in the window, +8.7% on 19-Jul-2022, is the Haleon demerger reprice*** *— GSK separated its consumer-health arm (Panadol, Sensodyne, Advil) into Haleon on 18-Jul-2022, the biggest European listing in a decade, and the remaining biopharma-only GSK repriced the next session.* ***The Zantac litigation then whipsawed the stock through a clean arc:*** *a -6.7% panic on 11-Aug-2022 as fears of ranitidine/NDMA cancer liability wiped ~GBP 30B off GSK, Haleon and Sanofi combined; a +7.8% relief on 6-Dec-2022 when the US federal MDL judge excluded plaintiffs' expert testimony and tossed ~50,000 claims; and a -8.7% setback on 3-Jun-2024 when a Delaware court ruled ~70,000 state cases could proceed. The remaining large moves are earnings and macro: +8.2% on 5-Feb-2025 and +7.3% on 4-Feb-2026 (FY results), and -6.4% on 4-Apr-2025 (the pharma-tariff selloff that hit the whole sector). The asymmetry is the finding: GSK's largest moves are events — a demerger, litigation rulings, results, policy — not daily beta, consistent with a low-volatility (25.5%) defensive name whose ADF p-value of 0.744 marks it a near-random walk between catalysts. Day-one attributions are analyst reconstructions from the cross-asset context on each date, not company confirmations.*

## 7a · Option-implied-vol Monte Carlo & earnings calendar

The option-implied cone is the most honest forward-looking band here, and it disagrees with the bootstrap Monte Carlo in Section 7 — a gap worth stating. ATM implied volatility is **29.9% at ~12 months and 29.0% at ~6 months** — low, consistent with GSK's 25.5% realized vol and its defensive character. Because this cone is **driftless** by construction, its 12-month median lands at ~$49 with a 44% probability of finishing above spot (the sub-50% figure is the lognormal median drag, not a bearish view), and its 6-month median is ~$51 — essentially spot. The Section 7 bootstrap Monte Carlo, by contrast, resamples the last three years of returns and therefore **embeds the drift of a period in which GSK recovered from a $35 low to $60**, producing a 12-month median of $61 and a 75% probability of finishing higher. That $61 is substantially an artifact of the lookback window, not a market-implied expectation; the driftless null — approximately spot — is the appropriate anchor, and the honest statement is that a 30%-vol defensive pharma has a moderately wide but roughly centered 12-month distribution ($30 to $80 at the 5th/95th percentiles). The next scheduled catalyst is the 28-Oct-2026 Q3 print; for a diversified pharma, however, scheduled earnings are not the dominant volatility source — pipeline readouts (bepirovirsen filings, HIV LAI data) and policy/litigation headlines are, and those do not sit on the earnings calendar.

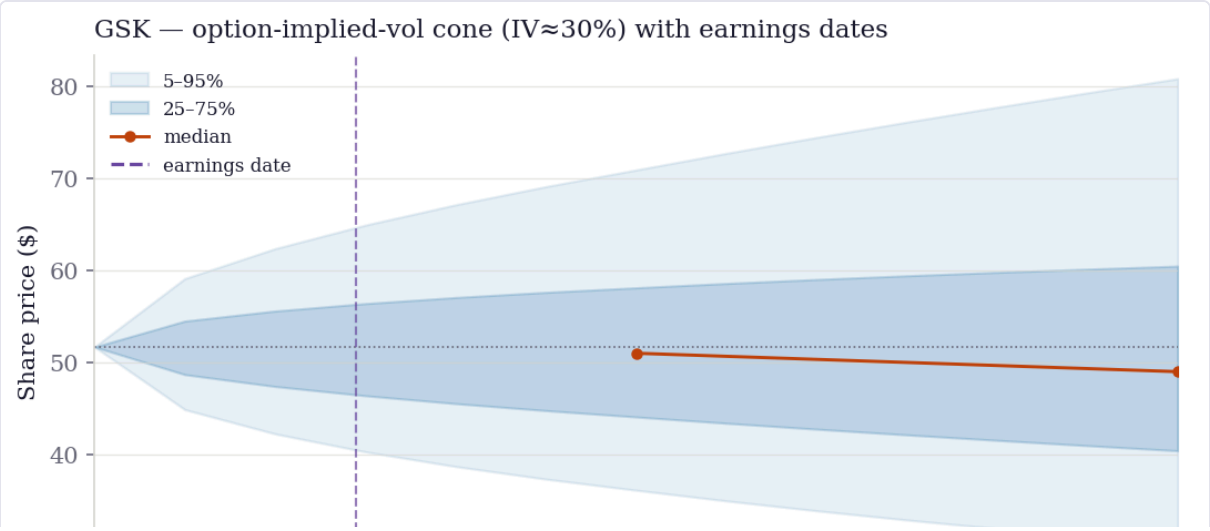





**Fig. 9.** Driftless lognormal price cone from ATM option-implied vol (~29.9% at ~12 months), with scheduled earnings dates marked.

| Horizon | P5 | P25 | Median | P75 | P95 | P(>current) |
|---|---|---|---|---|---|---|
| 6mo | $36 | $44 | $51 | $58 | $71 | 46% |
| 12mo | $30 | $41 | $49 | $61 | $80 | 44% |

*Table 16 — driftless option-implied-vol percentiles (contrast §7 bootstrap MC, which embeds recent drift). IV source: Yahoo Finance option chain via yfinance (ATM implied vol, ~6-month and ~12-month expiries); delayed quotes.*

**Earnings overlay.** Next report(s): 2026-10-28. Earnings prints are the largest scheduled volatility events. The IV cone above is a market-calibrated band; single-day gaps (see §7 events) can exceed it.

## 8 · Competitive landscape

GSK competes across four distinct arenas, and its position varies sharply by arena. In **HIV** it is the clear category leader through ViiV (~78.3% owned; Shionogi 21.7% after Pfizer exited in early 2026), with the dominant integrase-inhibitor franchise — but the competitive threat is generic dolutegravir post-2028-29 and, longer term, Gilead's lenacapavir-based long-acting regimens; GSK's answer is its own long-acting injectables (Cabenuva, Apretude) and an ultra-long-acting pipeline. In **vaccines** it is a global leader: Shingrix has no direct shingles competitor and a durable moat, while in RSV it is in a three-way fight with Pfizer (Abrysvo) and Moderna (mRESVIA) for a market the US ACIP capped by narrowing the older-adult recommendation. In **respiratory** it faces AstraZeneca and Sanofi/Regeneron (Dupixent) in immunology and biologics, where depemokimab's six-monthly dosing is its differentiation. In **oncology** it is a challenger, not a leader — the Blenrep relaunch and Jemperli put it back in the game after the 2022 Blenrep withdrawal, but it trails Merck, AstraZeneca and Bristol in scale. The defining strategic fact is structural: since demerging Haleon in July 2022, GSK is biopharma-only, so unlike the diversified J&J or the consumer-plus-pharma models it has no non-pharma ballast — it lives and dies by the pipeline. Against the large-cap-pharma lens it earns a top-of-group 22.2% ROIC (tying Merck, far above Pfizer 6.1% and Sanofi 6.7%) yet trades at the bottom of the group on forward earnings — the market's verdict that GSK's *current* quality is real but its *forward* growth is not yet trusted. Notably, GSK is absent from the obesity/GLP-1 arena entirely, which is both why it is cheaper than peers with that exposure and a competitive gap the pipeline does not address.

## 9 · SWOT

**STRENGTHS**

**WEAKNESSES**

- Top-of-peer-group returns: 22.2% ROIC against a 7.5% WACC (+14.7pt spread), tying Merck for the highest in the set and far above Pfizer (6.1%) and Sanofi (6.7%) — genuine value creation, not a value trap.
- Category leadership in HIV (ViiV, GBP 7.7B, +11%) and vaccines (Shingrix GBP 3.6B; a broad RSV/meningitis/shingles franchise, ~28% of sales) — two durable, high-margin moats.
- Cheap on the clean lens: 10.1x forward P/E vs a 14.6x peer median (-31%) with a 29.9% core margin (+110bps) and rising Specialty mix (Oncology +43%, Specialty +17%).
- Long-acting-injectable HIV (Cabenuva, Apretude +40-75%/qtr) actively converting the franchise off the vulnerable oral-dolutegravir base ahead of the 2028-29 cliff.
- Broad, diversified late-stage pipeline across respiratory, HIV, oncology, vaccines and infectious disease (depemokimab, bepirovirsen, Blenrep, Jemperli, maternal RSV, pentavalent meningitis).
- Defensive, well-covered 3.5% dividend (69% payout), low realized volatility (25.5%) and strong interest coverage (11.8x).

- Dolutegravir/HIV loss of exclusivity ~2028-29 hangs over the single largest earner — the market implies -4.4% durable growth as a result.
- Weak free-cash-flow conversion (14.6% FCF margin, 83% conversion) versus peers (~17%, ~100%) — the reason GSK screens 91% expensive on price/FCF.
- No GLP-1/obesity franchise — absent from the sector's biggest growth pool.
- Modestly higher leverage (net debt/EBITDA 1.4x, D/E 1.08) and tighter liquidity (current ratio 0.82) than the peer group.
- Oncology is a challenger, not a leader; the Blenrep relaunch carries execution risk after its 2022 US withdrawal.
- UK/FTSE domicile discount and a USD-ADR/GBP-accounts mismatch that distorts EV/EBITDA optics.

## OPPORTUNITIES

- Prove post-cliff growth: every point of durable growth GSK demonstrates is worth several turns of multiple (19x warranted at 3% growth vs 10.1x actual).
- Vaccines expansion: Shingrix into new geographies, a first pentavalent meningitis vaccine, and a maternal-RSV Phase III that opens a new indication.
- Oncology ramp: Blenrep guided a key 3-4-year growth driver, plus Jemperli label expansions into earlier lines and new tumours.
- Bepirovirsen as a potential first functional cure for chronic hepatitis B (Phase III positive, filings from Q1 2026) — a genuinely novel franchise.
- Ultra-long-acting / antibody HIV (twice-yearly regimens) to defend and extend the franchise past the dolutegravir cliff.
- Re-rating toward peer multiples as the Zantac

## THREATS

- A steeper or earlier HIV cliff, or faster uptake of competing long-acting regimens (Gilead lenacapavir), eroding the franchise before the LAI conversion matures.
- US drug-pricing policy — IRA negotiation, most-favoured-nation pricing and pharma tariffs (which drove the -6.4% April-2025 move) — compressing the highest-margin geography.
- Zantac residual: the federal MDL dismissal is on appeal and non-participating/qui-tam claims remain.
- RSV class disappointment: a narrowed ACIP recommendation and three-way competition capping Arexvy.
- Pipeline setbacks — any high-profile Phase III failure (bepirovirsen, depemokimab expansions, HIV LAI) would undercut the entire cliff-defense thesis.

overhang fully clears and cash conversion improves — the forward-P/E lens implies ~$57-61 at peer parity.

- Persistent UK-listing discount keeping the multiple structurally below US peers regardless of delivery.

## 10 · Company financial health

GSK's balance sheet is adequate rather than pristine, and free cash flow is the genuine soft spot. On **leverage and liquidity** it runs modestly heavier than the peer group: net debt/EBITDA of 1.4x (lease-inclusive) versus a 1.2x peer median, debt-to-equity of 1.08 versus 0.76, and a current ratio of 0.82 versus 1.14 — tighter, but backstopped by strong 11.8x interest coverage (versus 9.8x) and a lower, well-covered payout (69% of net income versus 97.5%), so this is a cushion question, not a solvency one. On **cash generation the read inverts and it is the weakness:** a 14.6% FCF margin and 83% FCF conversion both trail the peer medians (17.2% and ~100%), the direct cause of GSK's 91% 'expensive' reading on price/FCF and a reason the earnings-cheapness does not fully translate to cash-cheapness. **Two data caveats govern this table.** First, currency: GSK's figures are GBP while the US peers are USD and Sanofi is EUR, so only the *ratio* columns (current, quick, net-debt/EBITDA, D/E, coverage, FCF margin, payout) are comparable across names — the absolute cash/debt/FCF columns, shown with a '$' glyph, are in each company's own currency and must not be compared directly. Second, consolidation: GSK's figures include 100% of the 78.3%-owned ViiV, modestly overstating attributable cash flow.

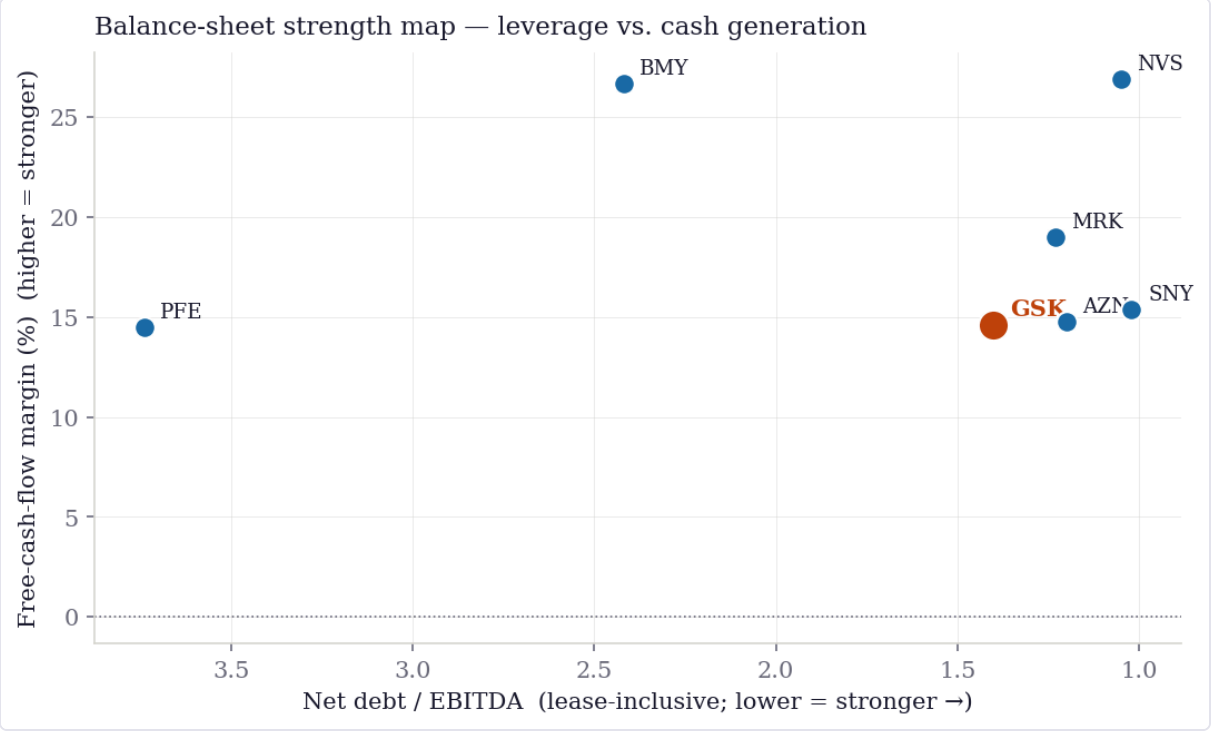



**Fig. 10.** Balance-sheet strength map — net debt/EBITDA (lease-inclusive) vs FCF margin; GSK highlighted.

| Company | Current | Quick | Cash | Fin. debt | Leases | ND/EBITDA (incl lease) | ND/EBITDA (ex-lease) | D/E | Int. cov |
|---|---|---|---|---|---|---|---|---|---|
| GSK GSK | 0.82 | 0.54 | $3.15B | $16.89B | $0.83B | 1.4× | 1.32× | 1.08 | 11.8× |

| | | | | | | | | | |
|---|---|---|---|---|---|---|---|---|---|
| AZN AZN | 0.94 | 0.72 | $5.71B | $27.35B | $1.8B | 1.2× | 1.11× | 0.6 | 8.7× |
| BMY Bristol Myers Squibb | 1.26 | 1.14 | $10.21B | $45.11B | $2.03B | 2.42× | 2.29× | 2.55 | 5.9× |
| MRK Merck & Co. | 1.54 | 1.3 | $14.56B | $49.34B | — | 1.23× | 1.23× | 0.94 | 16.5× |
| NVS NVS | 1.12 | 0.89 | $11.44B | $33.46B | $1.92B | 1.05× | 0.96× | 0.77 | 15.3× |